Behavioral Health Crisis

UP-17 | Universal Patient Care

Quick flow

01 Scene Safe? [EMT / AEMT / Paramedic]

Stage or withdraw when unsafe • request law enforcement and sufficient resources • identify weapons or hazards

02 Medical Emergency? [EMT / AEMT / Paramedic]

Airway • breathing • circulation • vitals • SpO_2 • glucose • temperature • trauma • neurologic and toxicologic causes

03 Assign BARS [EMT / AEMT / Paramedic]

1-3: altered mental status/overdose • 4: cooperative • 5: de-escalate • 6-7: UP-18 combined agitation pathway

04 Assess Safety Risk [EMT / AEMT / Paramedic]

Suicide • homicide • plan • intent • access to means • prior attempts • command hallucinations • ability to protect self

05 De-escalate [EMT / AEMT / Paramedic]

One calm speaker • reduce stimulation • maintain personal space and exit • acknowledge emotions • offer safe choices

06 Immediate Danger? [EMT / AEMT / Paramedic]

Direct observation • remove accessible hazards when safe • request authorized custody assistance • do not rely on a safety contract

07 Safe Disposition [EMT / AEMT / Paramedic]

Emergency department for medical risk, self-harm, restraint, or sedation • alternative destination only under approved policy

Overview

- Treat behavioral symptoms as a possible medical emergency until reversible medical, traumatic, toxicologic, and environmental causes have been assessed.
- The Behavioral Activity Rating Scale (BARS) measures current activity and agitation; it does not determine suicide risk or decision-making capacity.
- Protect the patient, public, and responders while preserving dignity and using the least restrictive effective intervention.

Provider Actions

Use explicit provider-level actions without substituting a generic scope statement.

EMT

- Establish scene safety, request law enforcement or additional personnel when indicated, and do not enter or remain in an unsafe environment.
- Assess airway, breathing, circulation, mental status, trauma, oxygenation, glucose, temperature, medications, substance exposure, and focused neurologic findings.
- Assign and document BARS, complete the suicide/homicide safety assessment, begin verbal and environmental de-escalation, and maintain direct observation when risk is present.
- Provide indicated BLS treatment, obtain two complete vital-sign sets when feasible, and transport to the appropriate emergency destination.

AEMT

- Perform all EMT care. Establish IV/IO access only when a medical condition, poor perfusion, significant overdose, or anticipated emergency treatment creates a clinical indication.
- Administer dextrose, naloxone, or IV fluid only when indicated under the applicable Claiborne medical, toxicology, or shock protocol; do not medicate solely for a psychiatric diagnosis.
- Continue physiologic monitoring, direct observation, and reassessment during transport.

Paramedic

- Perform all prior care. Obtain and interpret a 12-lead ECG, use $EtCO_2$, and provide advanced airway or cause-specific treatment when the presentation or intervention indicates.
- For BARS 6-7, transition to UP-18 Behavioral Agitation and Severe Agitation. Use the severe-agitation branch when delirium, hyperthermia, unusual strength, or prolonged struggle is present.
- Agitation medication and dosing are contained only in the combined UP-18 protocol. UP-17 contains no standing-order sedative treatment.
- After restraint or medication, provide continuous airway, ventilation, SpO_2, $EtCO_2$, and cardiac monitoring as required by UP-18.

BARS Routing

BARS standardizes observation and reassessment but never replaces medical evaluation or suicide-risk assessment.

EMT

- BARS 1: difficult or unable to awaken. Treat as an airway, altered mental status, overdose, or other medical emergency.
- BARS 2: asleep but responds normally to verbal or physical stimulation. Evaluate for medication, alcohol, overdose, hypoglycemia, or another medical cause.
- BARS 3: drowsy or appears sedated. Evaluate altered mental status, monitor airway and ventilation, and consider overdose or other CNS depression.
- BARS 4: quiet and awake with normal activity. Continue the complete behavioral, suicide/homicide, capacity, and medical assessment.
- BARS 5: increased verbal or physical activity but not disruptive. Continue UP-17, reduce stimulation, and attempt verbal de-escalation.
- Document the initial score and repeat BARS after de-escalation or any change in condition.

AEMT

- BARS 6: extremely or continuously active and disruptive but not violent. Request sufficient resources, continue de-escalation when safe, and initiate UP-18.
- Do not allow vascular access or nonessential procedures to provoke additional struggle or delay safe transport.

Paramedic

- BARS 7: violent with immediate danger and restraint required. Initiate the UP-18 safety and restraint pathway without delaying for nonessential assessment.
- Use the UP-18 severe-agitation branch when agitation includes delirium, hyperthermia, unusual strength, pain tolerance, or continued/prolonged struggle.
- Repeat and document BARS after physical restraint, medication, and each clinically important change.

Suicide and Violence Safety Assessment

A calm or cooperative patient may remain at high risk; BARS does not clear a patient for refusal or release.

EMT

- For medically able patients age 8 years and older, use the ASQ framework: recent wish to be dead; belief that self or family would be better off if dead; recent thoughts of suicide; prior suicide attempt; and current thoughts of suicide.
- Ask directly about plan, intent, timing, access to medications or weapons, preparations already made, previous attempts, recent self-harm, homicidal thoughts, command hallucinations, substance use, and ability to remain safe.
- If feasible and developmentally appropriate, interview pediatric patients privately for part of the assessment, then obtain collateral history from the parent or guardian.
- Any current suicidal intent, credible homicidal threat, recent serious attempt, dangerous command hallucination, or inability to protect oneself requires direct observation and emergency evaluation.
- Do not leave a high-risk patient unattended and do not use a verbal or written safety contract as proof of safety.

AEMT

- Maintain continuous observation and repeat mental-status, vital-sign, and risk assessment after any medical treatment or change in behavior.

Paramedic

- Integrate the safety assessment with medical findings and decision-making capacity. Contact Medical Control whenever risk, capacity, custody, or destination is uncertain.

De-escalation and Restraint Safety

Use restraint only to prevent harm or permit medically necessary assessment, treatment, and transport.

EMT

- Use one calm speaker, open-ended questions, active listening, emotional acknowledgment, nonthreatening posture, personal space, reduced noise and spectators, and reasonable choices.
- Maintain a clear exit and avoid confrontation, sudden movements, deceptive promises, crowding, or arguing about delusions.
- Do not use prone restraint, hog-tying, neck or chest compression, or any position or device that compromises airway, breathing, circulation, or rapid release.

AEMT

- Assess respiratory and hemodynamic status and the neurovascular status of restrained extremities as soon as safely possible and at recurring intervals.

Paramedic

- Physical or pharmacologic restraint requires the UP-18 pathway, appropriate physiologic monitoring, recurring reassessment, and transport to an emergency department.
- EMS medications must never be administered solely to facilitate arrest or law-enforcement custody.

Tennessee Emergency Detention and Capacity

Coordinate emergency custody through the authorized Tennessee process while continuing necessary medical care.

EMT

- Request law enforcement or another authorized professional when mental illness or serious emotional disturbance creates an imminent substantial likelihood of serious harm.
- EMS personnel do not independently initiate a Tennessee emergency mental-health detention solely by virtue of EMS licensure.

- Maintain observation and do not abandon a patient who presents an immediate safety threat or cannot make an informed decision.

AEMT

- Continue medically necessary care and transport while the authorized officer, physician, psychologist, or commissioner-designated professional manages the custody process.

Paramedic

- Assess whether the patient can understand relevant information, appreciate the condition and consequences, reason about options, and communicate a stable choice.
- A psychiatric diagnosis, unusual belief, intoxication, or refusal alone does not automatically establish incapacity; document the specific functional impairment when capacity is absent.
- Contact Medical Control and request authorized custody assistance whenever capacity or immediate risk is uncertain.

Acute Dystonic / Extrapyramidal Reaction

Recognize medication-induced dystonia as a medical condition that may be mistaken for behavioral illness.

EMT

- Suspect an acute dystonic reaction with involuntary facial, jaw, neck, trunk, or extremity spasm after an antipsychotic or other dopamine-blocking medication.
- Assess for tongue, pharyngeal, or laryngeal involvement and support airway and ventilation immediately when present.

AEMT

- Recognize dystonia, assess and support airway/ventilation, and request Paramedic care. AEMT diphenhydramine for dystonia is not enabled pending specific current Tennessee indication/route authority, competency and signed local orders.

Paramedic

- Adult: diphenhydramine 50 mg IV/IO/IM.
- Pediatric patient younger than 16 years: diphenhydramine 1 mg/kg IV/IO/IM; maximum 50 mg.
- Monitor airway, SpO_2, vital signs, and cardiac rhythm after parenteral medication and reassess the muscular symptoms.
- Perform all prior care. Treat airway compromise immediately and evaluate for serotonin syndrome, neuroleptic malignant syndrome, seizure, tetany, toxic exposure, or another cause when findings are atypical.

Indications

- Suicidal thoughts, suicide attempt, self-harm, homicidal thoughts, psychosis, hallucinations, delusions, severe anxiety, mania, depression, or another behavioral-health crisis.
- Agitation, threatening behavior, impaired judgment, inability to protect oneself, or concern for danger to the patient or others.
- Behavioral change that may be caused by a medical, traumatic, toxicologic, medication-related, or environmental emergency.

Assessment | Medical and Traumatic Causes

- Assess airway, ventilation, oxygenation, circulation, complete vital signs, glucose, temperature, pupils, focused neurologic status, trauma, pain, pregnancy when applicable, medication adherence or changes, substance exposure, and withdrawal.
- Consider hypoxia, hypoglycemia or hyperglycemia, stroke, seizure or postictal state, head injury, shock, sepsis, hyperthermia, intoxication, overdose, withdrawal, medication reaction, endocrine/metabolic disease, dementia, and delirium.

- Obtain a 12-lead ECG when overdose, stimulant use, syncope, chest discomfort, dyspnea, palpitations, abnormal pulse, electrolyte disorder, QT-prolonging medication, or parenteral agitation medication is suspected or used.
- Use $EtCO_2$ for hypoventilation, suspected overdose, significant altered mental status, restraint, or pharmacologic management.

Assessment | Behavior and Safety Risk

- Assign BARS from 1 through 7 and document objective observed behavior rather than conclusory labels such as uncooperative, crazy, or violent without supporting facts.
- Ask directly about suicidal and homicidal thoughts, plan, intent, access to means, prior attempts, self-harm, command hallucinations, substance use, supports, and ability to remain safe.
- Obtain collateral information from family, caregivers, witnesses, law enforcement, medication containers, and prior records when available, while maintaining patient dignity and necessary privacy.
- Identify abuse, neglect, exploitation, domestic violence, sexual assault, trafficking, or an unsafe living environment and follow the applicable reporting protocol.

Assessment | Capacity and Reassessment

- Assess whether the patient understands the situation and proposed care, appreciates likely consequences, can reason about options, and communicates a stable choice.
- Obtain two complete vital-sign sets when feasible. Reassess high-acuity, restrained, sedated, or clinically unstable patients at least every 3-5 minutes.
- Repeat BARS and the focused medical, airway, perfusion, neurologic, and safety assessment after every intervention or significant change.

Treatment steps

- Ensure scene safety; stage or withdraw when unsafe and request law enforcement and sufficient personnel before contact or intervention.
- Perform the medical screen and immediately treat hypoxia, hypoventilation, hypoglycemia, opioid respiratory depression, shock, hyperthermia, trauma, seizure, stroke, overdose, or another identified emergency under the applicable Claiborne protocol.
- Assign BARS and route BARS 1-3 to altered mental status/overdose evaluation, BARS 4 to the cooperative behavioral assessment, BARS 5 to de-escalation, and BARS 6-7 to the appropriate branch of the combined UP-18 protocol.
- Complete the suicide/homicide assessment. Maintain direct observation and remove accessible dangerous objects when this can be done safely.
- Attempt verbal and environmental de-escalation before restraint whenever the situation permits.
- When an imminent substantial likelihood of serious harm exists, request an authorized Tennessee custody professional and continue medically necessary care and safe transport.
- Use UP-18 for any physical or pharmacologic restraint, including severe agitation with delirium or hyperthermia. Never restrain or transport a patient prone.
- Transport to an emergency department for medical abnormality, injury, overdose, delirium, recent self-harm, current suicide or homicide risk, dangerous psychosis, restraint, or pharmacologic management.
- Use a psychiatric or crisis alternative destination only under a separately approved destination policy and only after required medical and safety screening criteria are met.
- A patient with intact capacity and no identified immediate danger may use the standard refusal process. Contact Medical Control whenever capacity or risk is uncertain and document a clear safety and follow-up plan.

Medications

Diphenhydramine

Adult: 50 mg IV/IO/IM. Pediatric patient younger than 16 years: 1 mg/kg IV/IO/IM; maximum 50 mg.

- Paramedic.
- Indication: acute dystonic or extrapyramidal reaction.
- Assess for airway involvement and monitor after administration.

Warnings / precautions

- Diphenhydramine - dystonia under UP-17: Paramedic-only administration until specific current Tennessee AEMT authority for this indication and route, applicable competency and signed local orders are documented. During that interval, the AEMT action is recognition, airway assessment/support and requesting Paramedic care. Do not infer dystonia permission from allergy treatment or use online Medical Control to expand scope. Use the Paramedic dose specified here.
- A calm, cooperative, or BARS 4 patient may still have imminent suicide or homicide risk.
- Never assume a psychiatric diagnosis until medical, traumatic, toxicologic, medication-related, and environmental causes have been assessed.
- Do not leave a high-risk patient unattended or rely on a verbal or written safety contract.
- Do not use prone restraint, hog-tying, neck or chest compression, or a restraint that cannot be rapidly released for airway, breathing, or circulatory compromise.
- EMS medication must not be used solely to facilitate arrest or law-enforcement custody.
- Any patient receiving physical restraint or pharmacologic management requires continuous observation, physiologic monitoring, recurring reassessment, and emergency-department transport.
- Do not place a psychiatric-only patient at an alternative destination unless a separately approved destination policy is active and all criteria are met.

Clinical pearls

- Ask directly about suicide and homicide; asking does not create suicidal thoughts and may identify concealed risk.
- BARS is a rapid observational communication tool, not a diagnosis, suicide-risk scale, capacity assessment, or automatic medication order.
- Medical causes are especially important with new behavioral change, atypical age of onset, abnormal vital signs, fluctuating attention, focal neurologic findings, trauma, or no established psychiatric history.
- One calm speaker and fewer stimuli often work better than multiple responders issuing simultaneous commands.
- Current suicidal thoughts, a feasible plan with access to means, a recent attempt, dangerous command hallucinations, or inability to protect oneself requires immediate safety precautions and emergency evaluation.
- If law-enforcement restraints that require a key must remain, law enforcement and the key must accompany the patient; transition to the least restrictive safe restraint when feasible.
- Exact patient statements are more clinically useful than paraphrases when documenting threats, hallucinations, intent, or refusal.

Special populations | Pediatrics

- Pediatric patients are younger than 16 years and medication doses use actual or length-based weight.
- The ASQ framework may be used for medically able patients age 8 years and older. Suspected suicide risk in a child younger than 8 years requires comprehensive emergency mental-health evaluation rather than relying on a screening score.

- When feasible and developmentally appropriate, ask the safety questions privately for part of the encounter and then obtain collateral information from the parent or guardian.
- Evaluate for abuse, neglect, exploitation, bullying, trafficking, medication ingestion, developmental disability, and an unsafe home environment.

Special populations | Law-Enforcement Custody

- A patient restrained by law enforcement remains an EMS patient when medical assessment or treatment is provided; EMS advocates for airway, breathing, circulation, positioning, monitoring, and dignity.
- Law enforcement should accompany a patient when law-enforcement restraints must remain during ambulance transport, and the required release key must be immediately available.
- Do not transport with hands restrained behind the back when that position interferes with assessment, monitoring, airway protection, or safe positioning.

Special populations | Refusal / Non-Transport

- Do not permit refusal based solely on a calm appearance, a low BARS score, denial after a documented threat, or a verbal safety contract.
- Current suicidal intent, credible homicidal threat, recent serious attempt, dangerous command hallucinations, inability to protect oneself, impaired capacity, medical instability, restraint, or sedation requires emergency evaluation and safe transport.
- A patient with intact capacity and no identified immediate danger may refuse only under the standard Refusal / Non-Transport protocol. Contact Medical Control whenever capacity or risk is uncertain.
- Document capacity, risk assessment, exact statements, collateral information, alternatives offered, risks explained, Medical Control or law-enforcement involvement, responsible support person, crisis resources, and return precautions.

Source and references

Tennessee EMS BLS/ALS State Protocol Guidelines, September 2025. | Tennessee Code Annotated §§ 33-6-401 through 33-6-404 and § 33-6-501 - emergency detention and substantial likelihood of serious harm. | National Institute of Mental Health - Ask Suicide-Screening Questions (ASQ) Toolkit for youth and adults. | National Association of EMS Physicians and partner organizations - Clinical Care and Restraint of Agitated or Combative Patients by EMS Practitioners, 2021. | Claiborne EMS UP-18 Behavioral Agitation and Severe Agitation. | North Carolina College of Emergency Physicians UP-17 Behavioral Health Crisis source protocol retained for historical comparison.

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